

Preparing to Establish Needs



AMENDMENT

In June 2024, text amends have been made to Section 5, Arranging the Assessment and Preparing the Person. These amends have been made to reflect feedback received following a full tri.x legal review of the Care Act 2014 Resource chapter 'Carrying Out an Assessment'.

[June 13, 2024](#)

1. Using this Procedure

This procedure should be used by anyone that has been allocated or asked to carry out any process that establishes the eligible needs of a person or carer under the Care Act, including any formal assessment or reassessment of need.

2. Letting the Person/Carer know you will be Carrying out an Assessment

2.1 General Guidance

You should contact the person/carers as soon as possible to let them know that you will be carrying out the assessment, even if you are not able to make the arrangements to do so yet.

The method of communication should reflect that requested by the person/carer and any specific communication needs they may have. For the purposes of the Care Act communication about the assessment is subject to the same requirements as the provision of information and advice, and the duty to make it accessible therefore applies equally.

See: [How to Provide Information and Advice](#) to read more about how to provide information in an accessible way under the Care Act.

In all cases where communication has been provided by telephone a follow up letter confirming the conversation and outcome should be sent to the person/carer as a formal record.

2.2 Where the person lacks capacity

If the person lacks capacity you should let an appropriate person know that you will be carrying out the assessment. Where available this should be a person legally authorised to represent them (for example a Court appointed Deputy or an Attorney) or an independent advocate. If the person lives in a care home this could also be the manager of the home.

2.3 Arranging an assessment

If you are able to make arrangements for the assessment to be carried out see [Section 5, Arranging the Assessment and Preparing the Person/Carer](#).

2.4 If you are unable to arrange the assessment yet

If you are not able to arrange the assessment at this first contact you should:

- a. Be satisfied that the delay is not going to increase the risk of deterioration in need or circumstances;
- b. Be satisfied that the delay is not going to increase the risk of abuse or neglect;

- c. Provide any information or advice that may be beneficial at that time;
- d. Be satisfied that urgent or interim support is not required;
- e. Be satisfied that the person/carer understands the reason for the delay;
- f. Agree a proposed timeframe for the assessment to be carried out and further contact to be made; and
- g. Advise the person/carer what to do should their situation change.

An element of monitoring should be incorporated into any delayed process to ensure that you remain aware of the person's/carer's situation and are able to respond appropriately to any changes or need to re-prioritise their assessment.

If the timeframe for assessment agreed with the person/carer changes you should let them know as soon as possible. If you experience difficulties prioritising your own work you should seek support from your line manager.

If the person/carer is not happy about any delays and you are satisfied that you are taking all reasonable steps to make arrangements and reduce risk you must make them aware of their right to complain.

3. Consent, Refusal and Engagement Difficulties

3.1 Where the person/carer does not consent to the assessment

A carer's assessment can only be carried out if the carer it relates to has given their consent.

The assessment of a person with Care and Support needs can only be carried out if the person it relates to has given their consent *unless*:

- a. The person lacks capacity to refuse the assessment and you are satisfied that carrying out the assessment would be in their best interests; or
- b. The person is experiencing, or is at risk of, abuse or neglect.

Where consent is not given you should:

- a. Confirm to the person/carer that the assessment is not being carried out on the basis that they have refused it;

- b. Provide any information and advice about adult Care and Support as appropriate and relevant;
- c. Provide any information and advice that will reduce, delay or prevent the need for Care and Support/Support;
- d. Let the person/carer know they have the right to an assessment at any point in the future on the appearance of need alone;
- e. Provide information about how the person/carer can request an assessment in the future; and
- f. Where there is a need to monitor the situation, agree how this will happen; and
- g. Where the situation is to be monitored, explain that the Local Authority will offer the person/carer another assessment at any point that it feels this would be beneficial to them.

Where a person with care and support needs has a carer the duty to assess the carer's needs still applies, unless the carer also refuses the assessment.

Where communication about refusal is provided by telephone a follow up letter confirming the conversation and outcome should be sent to the person/carer as a formal record.

If no monitoring activity has been agreed you should:

- a. Make a proportionate record of the person's/carer's refusal and the action you have taken;
- b. Notify the person who requested the assessment (if this was not the person/carer); and
- c. Proceed to close the case.

3.2 Consent and Capacity

If there are concerns that a person with care and support needs may lack capacity to consent to the assessment then a proportionate mental capacity assessment **must** be carried out to determine whether this is the case. This can be carried out by a person other than the assessor if they have the necessary skills to do so.

If the person has capacity to consent following the mental capacity assessment their consent must be obtained before carrying out the assessment.

If the person lacks capacity to consent following the mental capacity assessment then a Best Interest Decision must be made to confirm that carrying out the assessment will be in their Best Interests. This decision should be made by the assessor as the decision whether to assess or not rests with them.

See the [Mental Capacity Act 2005 Resource and Practice Toolkit](#), with guidance about assessing capacity and making best interest decisions.

3.3 Assessment when the person/carer has substantial difficulty engaging

If a person/carer has substantial difficulty being involved in any Care and Support process you must take all reasonable steps to ensure their involvement.

You must:

- a. Ensure that you have provided information in an accessible way, or that the person/carer has an appropriate person to support them to understand it;
- b. Arrange to carry out the assessment in an appropriate format so that it is accessible. This is likely to be face to face, unless the person's/carer's difficulty arises when engaging in face to face communication;
- c. Consider whether the person/carer has an appropriate person to support their involvement and, if not, whether the advocacy duty applies.

See: [Using Independent Advocacy](#), which includes guidance on how to establish whether a person/carer needs an advocate and how to make a referral.

3.4 Refusal of an assessment by the Local Authority

The only ground upon which the Local Authority can lawfully refuse any assessment is where there is **no** appearance of need. It does not matter whether the appearance of need is likely to prove eligible after any assessment. As such, it is unlikely that many people who request an assessment will be refused.

If you do feel that an assessment is not appropriate you should discuss this with a manager before refusing it as you must be sure that the decision to refuse is legally sound.

If any assessment is refused by the Local Authority you must confirm this in writing and explain to the person/carer:

- a. The reason the assessment has been refused;
- b. Information and advice about what can be done to prevent or reduce the development of needs for Care and Support in the future; and
- c. What to do if their needs change in the future.

3.5 Notifying others that an assessment has been refused

If the assessment was requested by anyone other than the person/carer and it has been refused you should notify the person who requested it so that they can make any arrangements to discuss other options with the person/carer. For example this could be:

- a. A carer (in the case of a person with Care and Support needs);
- b. A health professional;
- c. A social worker or an occupational therapist;
- d. A housing officer;
- e. A care provider.

3.6 Refused access to a person/carer requiring an assessment

Sometimes another person will obstruct you from carrying out an assessment with a person who has Care and Support needs (or a carer with Support needs). This can happen at any point from initial contact, to arranging an assessment, to carrying out an assessment. You should establish whether:

- a. The person with Care and Support needs (or carer with Support needs) has asked the person to obstruct the assessment, and if so whether they have capacity to refuse the assessment (this can only be achieved through direct contact with the person/carers);
- b. The person obstructing the assessment is doing so out of concern for the person with Care and Support needs or carer (for example would the assessment process cause anxiety);
- c. The person with Care and Support needs is at risk of abuse or neglect.

Wherever possible you should provide information and advice relating to adult Care and Support and the assessment process to the person obstructing the assessment, to support them to understand the benefits and engage in the process.

If the person continues to obstruct the assessment, and you are not able to establish from the person with Care and Support needs or carer that this is at their request you must take action to ensure the assessment is carried out. By obstructing the assessment process the person is:

- a. Putting the person with Care and Support needs or carer at risk through lack of support;
- b. Preventing the Local Authority from fulfilling its duties under the Care Act to assess and meet eligible needs.

An assessment in this situation can be carried out based on the information available and through consultation with others. The advocacy duty should be considered and it is likely that a referral would be appropriate to ensure that the person's/carers' involvement in the process is ensured.

Any information gathered must be enhanced at such time when the person/carers is involved, and all information sharing should give regard to confidentiality.

Depending on the circumstances consideration should also be given to raising an adult safeguarding concern.

See [Safeguarding Adults](#).

3.7 Assessment when the person is at risk of abuse or neglect

Under the Care Act you have a duty to carry out the assessment if a person with Care and Support needs is at risk of abuse or neglect. This is regardless of whether they consent to it or not.

Where the person does not consent but there is a duty to assess you should:

- a. Explain to the person that you have a duty to assess;
- b. Support and encourage the person to be involved in the assessment; and
- c. Carry out the assessment to the best of your abilities while ensuring their involvement as much as possible.

4. Gathering, Understanding and Using Available Information

4.1 What the Care Act says

The Care Act is clear that you should not ask the person/carer (or anybody that you consult with for the purpose of assessment) to repeat information that has already been made available to you unless there is a valid reason for doing so. For example, if you have not understood the information and want to clarify the meaning behind it then it may be appropriate to ask for it to be repeated, summarised or clarified.

Before continuing or beginning to carry out any kind of assessment you should take some time to read through the information that is already available for the purposes of:

- a. Understanding it; and
- b. Thinking about how it should inform the assessment process.

4.2 Sources of existing information

Existing sources of information may be available via:

- a. Referral forms;
- b. Recordings of any contact made;
- c. Letters, texts and e-mails;

- d. A confidential conversation with a colleague or manager who is familiar with the person/carer;
- e. Reports from professionals (for example a Speech and Language Therapist, Psychologist or Teacher);
- f. Records relating to current or historical safeguarding enquiries or concerns;
- g. Existing or historical mental capacity assessments; or
- h. Existing or historical assessment reports, review reports or Care and Support/Support Plans.

All information gathering and sharing should be carried out with regard to the Caldicott Principles, Data Protection legislation and local information sharing policies.

4.3 Other sources of information available

It may be appropriate and necessary for you to undertake an element of research in order to improve your own knowledge or awareness of a particular issue or health condition:

- a. By speaking with a colleague who possesses skills and knowledge in a particular area;
- b. By reading particular guidance or written resources about a particular medical condition;
- c. By accessing a reputable website for information and advice (for example Parkinson's UK, the British Institute for Learning Disabilities or MIND).

See: [National Organisations with Information and Advice Helplines](#) for details of some national organisations offering a specific information and advice helpline.

See: [National Contacts for Adult Care and Support](#) for a wider range of useful national contacts for adult Care and Support.

Need to know

Remember: see the [tri.x Resources](#) to access additional practice guidance that can support the processes of establishing needs, Care and Support Planning and review when a person has specific or complex needs.

4.4 Gathering Information and Consulting

You may need to speak to the person/carer in advance of arranging their assessment in order to gather or clarify any information.

You may also identify a benefit in gathering information and consulting with others. However, under the Care Act you are only permitted to consult with those people that the person/carer has consented to you consulting with (unless a person with Care and Support needs lacks capacity to provide consent when a decision can then be made in their best interests).

See: [Gathering Information and Consulting with Others](#).

5. Arranging the Assessment and Preparing the Person/Carer

Legally you must have regard to the following when arranging an assessment:

- a. The person's/carer's views and wishes about when and how the assessment is carried out, including who they would like to support them;
- b. The impact of any delay in assessment on their individual Wellbeing; and
- c. Whether any information and advice can be given to them that will prevent, delay or reduce the need for Care and Support/Support.

5.1 When to arrange the assessment

The Care Act statutory guidance states that 'an assessment should be carried out over an appropriate and reasonable timeframe taking into account the urgency of the needs or situation and consideration of any fluctuating needs'. This means that arbitrary timeframes (fixed timeframes that apply to all) should be avoided and a more flexible approach taken that reflects the needs of the person/carer and their situation.

The timing of the assessment should be agreed with the person/carer and anyone else who is to be involved in it.

You should also take into account the following:

- a. The level of risk;
- b. The level of need;
- c. Current support in place and the sustainability/effectiveness of this;
- d. The urgency;
- e. The likelihood of deterioration;
- f. The potential for fluctuation;
- g. Any clear indication of timeframe provided within any referral or recorded contact; and
- h. Whether the person/carer will need the support of an independent advocate and the time it may take to arrange this.

5.2 How to arrange the assessment

The method of communication should reflect that requested by the person/carer and any specific communication needs they may have. For the purposes of the Care Act communication about the assessment is subject to the same requirements as the provision of information and advice, and the duty to make it accessible therefore applies equally.

See: [How to Provide Information and Advice](#) to read more about how to provide information in an accessible way under the Care Act.

In all cases where communication has been provided by telephone a follow up letter confirming the conversation and outcome should be sent to the person/carer as a formal record.

5.3 What should be considered when arranging the assessment

The most important thing you must consider when arranging the assessment is how you will ensure the involvement of the person/carer. Some of the things you should think about include:

- a. Whether the person/carer will require independent advocacy;

- b. Whether the person/carer will find any assessment process to be emotionally difficult and what can be done to reduce their anxiety;
- c. The information the person/carer may need to prepare for the assessment;
- d. Whether the person/carer requires any support with communication (for example, will any assessment tool need to be adapted, will an interpreter be required or does a Speech and Language Therapist need to be involved);
- e. Whether the person/carer would like for anyone in particular to be involved in any assessment;
- f. Which environment would be best to meet in (if a meeting is to be arranged); and
- g. Whether the assessment needs to consider any physical needs the person/carer has for medication, rest or personal care.

When arranging the assessment you should also identify other people who may need to be a part of it. For example, a health professional or a service provider may need to be involved.

Need to know

Remember: see the [tri.x Resources](#) to access additional practice guidance that can support the processes of establishing needs, Care and Support Planning and review when a person has specific or complex needs.

5.4 If the person lacks capacity

If a person with Care and Support needs lacks capacity you should make arrangements with an appropriate person. Where available this should be a person legally authorised to represent them (for example a Court appointed Deputy or an Attorney) or an independent advocate. If the person lives in a care home this could also be the manager of the home.

5.5 Preparing the Person/Carer for Assessment

Under the Care Act you **must** provide information about the assessment process to the person (or their representative if they lack capacity)/carer as early as possible. This will help ensure their involvement in the assessment and any subsequent Care and Support processes. Wherever practical this

should be provided before the assessment process begins.

Any information should be provided in an accessible way for the person/carer who will be receiving it. In all cases where information has been provided by telephone a follow up letter confirming the information provided should be sent to the person/carer.

The information that should be provided is as follows:

- a. Information about what can be expected during the assessment process;
- b. The format that the assessment will take (e.g. telephone assessment, face-to-face assessment);
- c. The indicative timeframe for assessment (when will it begin and how long is it likely to take);
- d. The complaints process; and
- e. Information about possible access to independent advocacy.

You should also provide a list of any questions that you intend to ask of the person/carer during the assessment. This will help them to prepare for the assessment and to think about their Wellbeing, the impact of their needs on this and what outcomes they want to achieve.

5.6 Information about finances

Financial assessment is often a key point of anxiety for people and it is important that you are able to provide good information and advice (either directly or by supporting the person/carer to access it from an appropriate person or source).

Needs assessment

You must inform the person (or their representative if they lack capacity)/carer that a financial assessment will take place in due course and that the outcome of this will determine whether they will need to make a financial contribution to the cost of any support, services or equipment arranged.

Reablement

You must inform the person (or their representative if they lack capacity) that there is no charge attached to the reablement service for the first 6 weeks that it is provided, regardless of their financial circumstances. You should also explain that any equipment or aids provided to them to promote independence will also be non-chargeable up to the cost of £1000.

When you explain to the person that there will be no charge for reablement for the first 6 weeks you also need to explain what may happen financially after this point. Specifically you must explain that, subject to a financial assessment the person may be asked to make a contribution to:

- a. Any extended reablement support provided to them after the 6 week non-chargeable period; and
- b. Any Care and Support provided to them outside of the reablement support (either provided alongside reablement or after reablement has finished).

Occupational Therapy

You must inform the person (or their representative if they lack capacity) that:

- a. If, following assessment the Local Authority provides equipment to meet eligible needs this will be non-chargeable up to £1000, regardless of their financial circumstances;
- b. If the cost of any equipment is above £1000 then, subject to a financial assessment, they may be asked to make a contribution;
- c. If, following assessment the Local Authority agrees to carry out any major adaptations to their home they may have to make a financial contribution to the cost of the works, subject to a DFG application; and
- d. That if, at any point any other Care and Support is provided (for example urgent support) they may have to make a financial contribution.

It is your responsibility to familiarise yourself with local policy about charging for equipment over £1000 before confirming this to the person.

Guidance and tools

See: [Specific Requirements on the Provision of the Information and Advice around Finances](#) for guidance on the requirements of the Care Act.

See the [Financial Assessment and Charging FAQ Response Support Tool](#) for the answers to some frequently asked questions around financial assessment.

See the [Charging and Financial Assessment Procedure](#) for further guidance.

6. Using Independent Advocacy

If the need for independent advocacy has not already been established at contact or referral then this should become clear when either arranging the assessment or providing information about it. As soon as this is established you must consider whether the duty to make independent advocacy applies and, if so make the necessary arrangements.

See: [Using Independent Advocacy](#), which includes guidance on how to establish whether a person/carer needs an advocate, the different advocates that are available and how to make a referral.

7. Preparing Others for Involvement

7.1 In all cases

In some cases the only person who will need to be involved in the assessment will be the person with Care and Support needs (or carer with Support needs). In other cases more people will need to be involved because:

- a. In the case of an assessment for a person with Care and Support needs, they have a carer (carer's must be involved in the person's assessment);
- b. The person/carer has asked you to involve another person;
- c. The person/carer has consented to the involvement of another person;
- d. The person/carer will be using independent advocacy;
- e. A person with Care and Support Needs lacks capacity and a decision has been made in their best interests to involve another person; or
- f. The assessment is being carried out because of the risk of abuse and neglect and there is a need to involve others.

It is important that everyone who is to be involved in an assessment is aware of:

- a. The purpose of the assessment;
- b. The process of assessment; and
- c. Their role in any assessment.

The needs of others involved in an assessment should also be considered, but this should not be at the detriment of ensuring the person's/carer's own involvement.

If a person has a role in the assessment process but is not able to physically attend any planned meeting it is possible under the Care Act to consult with them separately and still include their views in any assessment and decision making processes.

7.2 Adult needs assessment

The Care Act is clear that a whole family approach to assessment should be taken wherever possible. This means that you should:

- a. Establish who relevant family members are; and
- b. Arrange for them to be involved in the assessment (if the person consents or lacks capacity and it is in their best interests to do so).

Relevant family members include anyone living with the person, including older adults and any children. It can also include family members who are not living with the person, but who are still involved or interested in the person's wellbeing.

Family members may have particular questions and require additional information to the general guidance above, which could include:

- a. Information relating to financial assessment;
- b. Information about mental capacity and best interests;
- c. Information about a Lasting Power of Attorney;
- d. Information about Appointeeship;
- e. Information about Deprivation of Liberty Safeguards; and

- f. Information about carers assessment.

7.3 Where the person has not consented for a carer to be involved

Under the Care Act carers have to be a part of the cared for person's assessment, even if the cared for person does not want them to be. This is because the Local Authority needs to:

- a. Understand the person's needs, how they are currently being met and whether this is appropriate and sustainable; and
- b. Identify and support carers.

When a person has not consented to the carer being a part of the assessment you should:

- a. Advise the person that you have a duty to involve the carer;
- b. Explain the benefits of the carer being involved; and
- c. Agree the most appropriate way to involve the carer (for example a separate meeting with the carer).

If the person has requested particular information not relating to needs is withheld from the carer, and they have capacity to do so normal confidentiality rules apply *unless* doing so would put the person (or another vulnerable adult or child) at risk of abuse or neglect.

7.4 Carers Assessment

The Care Act is clear that a whole family approach to assessment should be taken wherever possible. This means that you should:

- a. Establish who relevant family members are; and
- b. Arrange for them to be involved in the assessment **if the carer consents** to this.

Relevant family members include the cared for person, anyone living with the carer, including older adults and any children. It can also include family

members who are not living with the carer, but who are still involved or interested in the carer's wellbeing.

8. Deciding the Method of Assessment

8.1 The different methods of assessment available

The Care Act recognises a whole host of different methods of assessment, any of which could be appropriate so long as:

- a. The person's/carer's involvement is ensured by the method;
- b. The method is *appropriate* and *proportionate* to the needs and circumstances of the person/carer;
- c. The method will provide a full picture of the persons/carers needs in regard to the impact that those needs have on their Wellbeing; so that
- d. The Local Authority can provide an appropriate response at that time.

Possible assessment methods include:

- a. Face to face assessment;
- b. Telephone assessment;
- c. Online assessment;
- d. Combined assessment (with a carer);
- e. Joint or integrated assessment;
- f. Supported self assessment; and
- g. Delegated assessment

To access the Care Act definitions for each of the above assessment methods, see: [Methods of Assessment](#), part of the Care Act 2014.

8.2 Combining a needs assessment with a carers assessment

A needs assessment can be carried out under the Care Act as a combined assessment at the same time as any carers assessment, so long as:

- a. It is deemed appropriate to do so (for example both parties involvement can still be ensured and there is no conflict between the parties);
- b. Both the person and the carer agree to a combined assessment process and the sharing of information; or
- c. The person with care and support needs lacks capacity to agree but a best interest decision is made to carry out a combined assessment.

8.3 How to decide the method of assessment

When making a decision about the method of assessment you **must have regard** to:

- a. The wishes and preferences of the person/carers;
- b. The outcome the person/carers seeks from the assessment; and
- c. The severity and overall extent of the person's/carers' needs.

Some of the other factors that should be considered include:

- a. Availability of a particular assessment method;
- b. If a person with Care and Support needs has a carer, whether carrying out a combined assessment process would be beneficial.
- c. Whether there is a concern about the mental capacity of a person with Care and Support needs in relation to a particular decision to be made.
In this situation the Care Act requires you **must** carry out a face to face assessment;
- d. Whether the method of assessment chosen poses any challenges or risks for the person/carers;
- e. The specific communication needs of the person/carers (specifically whether they will be able to engage in the assessment method);
- f. The potential fluctuation of the persons/carers needs or situation; and

g. Any need for multidisciplinary working or assessment.

9. Tools to Establish Needs

Using a tool to support the process of establishing needs or formal assessment can be useful for all involved. If you are going to use a tool you should endeavour to provide this to the person/carer (or whoever will be supporting them) before the assessment is scheduled to take place.

See: [Tools and Practice Guidance to Establish Needs](#).

10. Joint Work

Sometimes there may be a clear benefit to a joint assessment with another service area, team or professional. The Care Act recognises this and permits the Local Authority to make any arrangements it deems appropriate in order to facilitate joint working with others.

10.1 The duty to co-operate

Where the Local Authority requests another party work jointly in some way to benefit the person with Care and Support needs (or carer with Support needs) that party has a duty to co-operate with the request (unless by doing so they will be prevented from carrying out their own duties under the Care Act or other legislation).

For further information about the duty to co-operate under the Care Act, see: [Co-Operation](#).

10.2 How to request joint work or assessment

Any decision to request joint work should be made with the person/carer (or their representative). Where a person with Care and Support needs is unable to provide consent to joint work decisions should be made in their best interests.

Joint work requests should be made in the manner preferred by the service, team or professional to which the request is being made. This may or

may not take the form of a referral.

The request should explain clearly the nature of the joint work required and any specific skills, knowledge and competence requirements to support allocation.

10.3 Responding to a request for joint work

If you have been asked to work jointly with a colleague in adult Care and Support or in another organisation (such as health or housing) you should contact the person you will be working jointly with to confirm your involvement and discuss the most effective way to work together. The things you should establish include:

- a. The work they are doing/will be doing/have done and whether they have any information that you need to know or can use to avoid duplication;
- b. Whether there are opportunities to co-ordinate systems and processes and, if so how this will be managed;
- c. What the expectations are in terms of joint-working (for example will you be expected to carry out a joint assessment, meet with the person/carer together, produce joint records or just consult and share information);
- d. What the anticipated outcome of the joint work is (for example joint funding of support, on-going joint-work to monitor);
- e. What does the person/carer know about the joint-work to be carried out (and if they don't know who and how should this be explained);
- f. Who will be the primary contact for the person/carer (or their representative) to go to with any queries; and
- g. Who will be responsible for communicating progress and decisions to the person/carer.

See: [Joint Work](#) for further practice guidance about effective joint working.

10.4 Specialist joint work or assessment

Some areas of joint work are specialist in nature. The procedures for these pieces of work can be found in the Specialist Procedures section. The

following are examples of the procedures that can be found there:

- a. NHS Continuing Healthcare;
- b. Continuity of Care;
- c. Cross Border Placement.

11. Preparing in an Urgent Situation

Sometimes the timeframe within which to arrange an assessment is not conducive to all of the considered planning and preparation as outlined in this procedure. For example, there may not always be the time to research and make the best use of available information or to consult with others before arranging to meet with the person/carer. However, even when the timeframe is short it is of paramount importance that you still:

- a. Take all available steps to ensure the involvement of the person/carer in any assessment;
- b. Consider consent and mental capacity;
- c. Provide independent advocacy where there is a statutory duty to do so;
- d. Prepare the person/carer as much as possible;
- e. Identify what existing information is important and understand it; and
- f. Know what you have not been able to do and ensure that at the earliest opportunity you make arrangements to do so (for example to consult with another professional or read a particular report).

If there are urgent needs you may need to arrange interim services to ensure that these are met during the assessment process. If this is the case, see: [Urgent or Interim Support](#).

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